

Name : VIPAN KUMAR GARG
Lab No. : 194176870
Ref By : Self
Collected : 20/8/2025 7:29:00AM
A/c Status : P
Collected at : PSC PANCHKULA HOME VISIT
206, SECTOR-20, PIN CODE NO -134116
PANCHKULA

Age : 68 Years
Gender : Male
Reported : 21/8/2025 3:52:55PM
Report Status : Interim
Processed at : LPL-CHANDIGARH LAB
Plot No. 17, Ground Floor, Industrial Area,
Phase-1, Backside Citroen Showroom,
Chandigarh-160002.



Test Report

Test Name	Results	Units	Bio. Ref. Interval
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MULTIPLE MYELOMA MONITORING PANEL

COMPLETE BLOOD COUNT

Hemoglobin (Oxyhemoglobin)	11.10	g/dL	13.00 - 17.00
Packed Cell Volume (PCV) (Calculated)	32.50	%	40.00 - 50.00
RBC Count (Electrical impedance/ Coulter Principle)	3.35	mill/mm3	4.50 - 5.50
MCV (Measured RBC histogram)	97.00	fL	83.00 - 101.00
Mentzer Index (Calculated)	29.0		
MCH (Calculated)	33.10	pg	27.00 - 32.00
MCHC (Calculated)	34.10	g/dL	31.50 - 34.50
Red Cell Distribution Width (RDW) (Measured RBC histogram)	16.10	%	11.60 - 14.00
Total Leukocyte Count (TLC) (Electrical impedance/ Coulter Principle)	3.50	thou/mm3	4.00 - 10.00

Differential Leucocyte Count (DLC)

Segmented Neutrophils (VCS Technology)	42.20	%	40.00 - 80.00
Lymphocytes (VCS Technology)	41.20	%	20.00 - 40.00
Monocytes (VCS Technology)	10.60	%	2.00 - 10.00
Eosinophils (VCS Technology)	5.40	%	1.00 - 6.00
Basophils (VCS Technology)	0.60	%	<2.00

Absolute Leucocyte Count

Neutrophils (Calculated)	1.48	thou/mm3	2.00 - 7.00
Lymphocytes (Calculated)	1.44	thou/mm3	1.00 - 3.00
Monocytes (Calculated)	0.37	thou/mm3	0.20 - 1.00



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Eosinophils (Calculated)	0.19	thou/mm3	0.02 - 0.50
Basophils (Calculated)	0.02	thou/mm3	0.02 - 0.10
Platelet Count (Electrical impedance/ Coulter Principle)	141	thou/mm3	150.00 - 410.00
Mean Platelet Volume (Measured Platelet histogram)	11.1	fL	6.5 - 12.0
There is mild leucopenia with neutropenia. Platelets are mildly decreased. Advised: Followup and clinical correlation Urgent review with Fresh EDTA sample if not correlating clinically.			

Comment

In anaemic conditions Mentzer index is used to differentiate Iron Deficiency Anaemia from Beta- Thalassemia trait. If Mentzer Index value is >13, there is probability of Iron Deficiency Anaemia. A value <13 indicates likelihood of Beta- Thalassemia trait and Hb HPLC is advised to rule out the Thalassemia trait.

Note

- As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood
- Test conducted on EDTA whole blood



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
ALBUMIN, SERUM (BCG)	4.38	g/dL	3.20 - 4.60
SERUM CALCIUM (Arsenazo III)	8.70	mg/dL	8.80 - 10.20



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
LDH;LACTATE DEHYDROGENASE, SERUM (IFCC)	146.00	U/L	120 - 246

Comments

Lactate dehydrogenase (LDH) is a nonspecific enzyme found in most organs. Highest concentrations are found in liver, heart, kidney and blood cells. LDH measurements are used in the diagnosis and treatments of liver diseases like Acute viral hepatitis, Cirrhosis & Metastatic carcinoma; Cardiac diseases like Myocardial infarction; Tumors of lungs / kidneys & Hematologic disorders like Megaloblastic anemia & Hemolytic anemia.



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**Test Report**

Test Name	Results	Units	Bio. Ref. Interval
SERUM UREA (Urease GLDH)			
Urea	39.00	mg/dL	17.00 - 49.00
BUN; Blood Urea Nitrogen	18.21	mg/dL	8.00 - 23.00



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Test Report

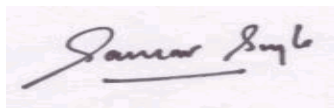
Test Name	Results	Units	Bio. Ref. Interval
SERUM CREATININE (Modified Jaffe, IDMS traceable)			
Creatinine	1.71	mg/dL	0.70 - 1.30
GFR Estimated	43	mL/min/1.73m2	>59
GFR Category	G3b		

Advise

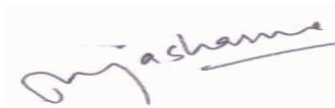
1. CKD Risk Map (Z1014)
2. Cystatin C, serum (B173)

Note

1. GFR, estimated (eGFR) calculated using the 2021 CKD-EPI creatinine equation and GFR Category reported as per KDIGO guideline 2012.
2. eGFR category G1 or G2 does not fulfil the criteria for CKD, in the absence of evidence of kidney damage



Dr. Gaurav Singla
MBBS MD-Pathology
Consultant Pathologist



Dr Pooja Sharma
MD, Pathology
Chief of Laboratory
Dr Lal PathLabs Ltd



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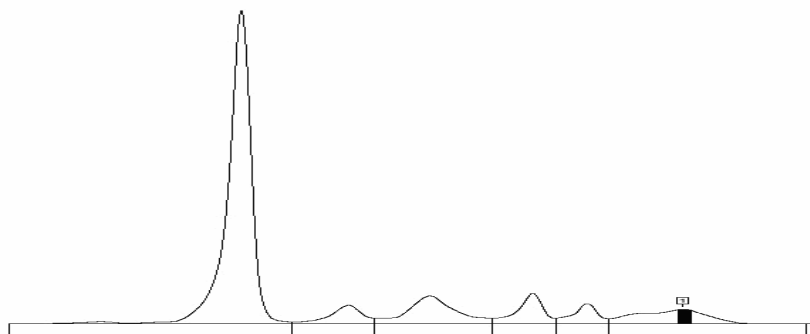
MULTIPLE MYELOMA MONITORING PANEL

PROTEIN ELECTROPHORESIS, SERUM

(Capillary Electrophoresis)

Protein, Total	6.30	g/dL	6.40 - 8.10
Albumin	4.01	g/dL	3.60 - 5.40
Alpha 1 globulin	0.33	g/dL	0.20 - 0.40
Alpha 2 globulin	0.72	g/dL	0.50 - 1.00
Beta 1 globulin	0.42	g/dL	0.50 - 1.10
Beta 2 globulin	0.27	g/dL	0.30 - 0.60
Gamma globulin	0.55	g/dL	0.70 - 1.50
A : G Ratio	1.75		0.90 - 2.00
M Spike	? 0.1	g/dL	

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Test Name	Results	Units	Bio. Ref. Interval
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Interpretation

Hypoproteinemia present.

Suspicious non-discrete protein band in gamma globulin region, possibly Monoclonal gammopathy.

Advised: Immunofixation electrophoresis (IFE) to confirm the presence of monoclonal gammopathy and identify its immunological nature.

Comments

Serum Protein electrophoresis (SPE) is used to identify patients with Monoclonal gammopathies (Multiple Myeloma (MM), Waldenstrom's macroglobulinemia, AL Amyloidosis as well as premalignant conditions Monoclonal gammopathy of unknown significance (MGUS) and Smoldering Myeloma) and other serum protein abnormalities (Nephrotic syndrome, Alpha 1 antitrypsin disorder, and inflammatory processes associated with infection, liver diseases & autoimmune disorder). Monoclonal gammopathies indicate clonal expansion of plasma cells or mature B cells. Monoclonal gammopathies are present in 8% of geriatric patients and require further evaluation for monitoring & prognosis. SPE can be used for monitoring response to therapy, a decrease or increase of M spike by 0.5 g/dl is considered significant change.

If SPE alone is used as initial diagnostic screen it will be able to detect approximately 88% of all MM, 66% of AL Amyloidosis and 56% of Light Chain Deposition Disease (LCDD) patients. SPE has good sensitivity in detecting intact monoclonal protein but has limited sensitivity in detecting monoclonal free light chains (FLC). Therefore, SPE alone will miss many patients that are only producing monoclonal FLC. These misses include many Light Chain MM, all Non Secretory MM and significant numbers of AL amyloidosis and LCDD patients. Combination of FLC assay with SPE & immunofixation improves the sensitivity to 99%. Thus, Multiple Myeloma screening panel which includes SPE, IFE and FLC should be done to screen patients with high clinical suspicion for MM.



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
BETA 2 MICROGLOBULIN, SERUM (CLIA)			
Beta-2 Microglobulin, Serum	10244	ng/mL	609.00 - 2366.00

Note

- False negative / positive results are observed in patients receiving mouse monoclonal antibodies for diagnosis or therapy.
- Beta 2 Microglobulin values regardless of levels should not be interpreted as absolute evidence of the presence or absence of disease. All values should be correlated with clinical findings and results of other investigations.

Comment

As per International Staging System for Multiple Myeloma the level of Beta 2 microglobulin can be used for staging the disease:

STAGE	BETA 2 MICROGLOBULIN LEVEL IN (ng/mL)
Stage I	<3500
Stage II	3500-5500
Stage III	>5500

Increased levels

- * Lymphoproliferative disorders like Multiple myeloma, B cell Lymphoma and Chronic Lymphocytic Leukemia.
- * Inflammatory disorders like Rheumatoid arthritis, SLE, Sjogren's syndrome and Crohn's disease.
- * Renal Dysfunction

Uses

- * Prognostic indicator of Multiple myeloma and other hematopoietic malignancies
- * An aid in the management of patients with Renal dysfunction and Rheumatoid arthritis
- * Staging of Multiple Myeloma

IMMUNOGLOBULIN PROFILE, SERUM



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
Immunoglobulin IgG, Serum (Immunoturbidimetry)	515.00	mg/dL	650.00 - 1600.00
Immunoglobulin IgM, Serum	<21.0	mg/dL	50.00 - 300.00
Immunoglobulin IgA, Serum	59.10	mg/dL	40.00 - 350.00

Comments:

Approximately 80% of serum immunoglobulin is IgG, 6% is IgM and 13% is IgA. High levels of IgM signify acute infections whereas IgG predominates in chronic infections. IgA is the predominant immunoglobulin in body secretions like saliva, sweat and colostrums.

Polyclonal increases are seen in:

- IgG: SLE, Chronic liver diseases, Infectious diseases and Cystic fibrosis
- IgM: Viral, bacterial and parasitic infections, Liver diseases, Rheumatoid arthritis, Scleroderma, Cystic fibrosis & heroin addiction
- IgA: Chronic liver diseases, Chronic infections, Autoimmune disorders, Sarcoidosis and Wiscott-Aldrich syndrome.

Monoclonal increases are seen in IgG & IgA Myelomas and IgM increases in cases of Waldenstroms macroglobulinemia

Decreased synthesis of IgG, IgM & IgA is seen in Congenital and Acquired Immunodeficiency diseases.

Decreased levels are seen in Protein losing enteropathies, Nephrotic syndrome and skin burns.





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Test Report

Test Name	Results	Units	Bio. Ref. Interval
KAPPA / LAMBDA LIGHT CHAINS, FREE, SERUM (Immunoturbidimetry)			
Kappa, Free Light Chain	38.71	mg/L	3.30 - 19.40
Lambda, Free Light Chain	16.58	mg/L	5.71 - 26.30
Kappa/Lambda(FLC), Ratio	2.335		0.26 - 1.65

Interpretation

KAPPA	LAMBDA	KAPPA / LAMBDA RATIO	REMARKS
Normal	Normal	Normal	Normal sample. If serum electrophoretic pattern is normal, it is unlikely that patient has Monoclonal gammopathy.
Normal	Normal	Low/High	Suggestive of Monoclonal gammopathy with Bone Marrow suppression
Low	Low	Normal	Suggestive of Bone Marrow suppression
Low	Low	Low/High	Suggestive of Monoclonal gammopathy with Bone Marrow suppression
High	High	Normal	Suggestive of : • Renal Impairment • Overproduction of Polyclonal free light chain from inflammatory conditions • Biclinal gammopathy of different free light chain types
High	High	Low/High	Suggestive of Monoclonal gammopathy with Renal Impairment

Note: International Myeloma Working Group (IMWG-2014) has updated the criteria for the diagnosis of Multiple Myeloma (MM) which includes clonal bone marrow plasma cells ($\geq 10\%$) or biopsy proven Plasmacytoma with one or more 'Myeloma defining event'. An elevated involved : uninvolved serum FLC ratio >100 (and involved FLC $\geq 100\text{mg/L}$ is now considered a 'Myeloma defining event'.

Comment

Measurement of free light chain (FLCs) aids in the diagnosis & monitoring of Multiple myeloma, Waldenstrom's macroglobulinemia, AL Amyloidosis & Light chain deposition disease. Serum concentration of FLCs is dependent upon the balance between production by plasma cells & their progenitors and renal clearance



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When there is increased polyclonal immunoglobulin production and/or renal impairment, concentration of both kappa & lambda light chains can increase by 30-40 fold but the Kappa / Lambda ratio remains unchanged. In contrast, tumor produces monoclonal excess of only 1 type of light chains, often with bone marrow suppression of alternate light chains resulting in abnormal kappa/lambda ratio.			

The combination of Serum protein electrophoresis (SPE) & Immunofixation (IFE) are highly sensitive but fail to identify a few patients with Light Chain Multiple Myeloma & all patients of Non Secretory Multiple Myeloma. Combination of free light chain assay with SPE & IFE improves the sensitivity to 99%.

Uses

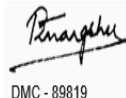
As per recommendations by IMWG:

- FLC, SPE and serum IFE for screening
- For diagnosis and prognosis of all patients with MGUS, Smoldering or active multiple myeloma, Solitary plasmacytoma and AL amyloidosis
- Serial measurements routinely in patients with AL amyloidosis and Multiple myeloma
- patients with oligosecretory disease
- In all patients who have achieved a complete response (CR) to assess for stringent CR



DMC - 67327

Dr Anjalika Goyal
MD, Biochemistry
Consultant Biochemist
NRL - Dr Lal PathLabs Ltd



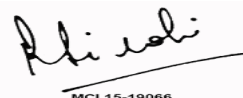
DMC - 89819

Dr Himangshu Mazumdar
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DMC - 9550

Dr Nimmi Kansal
MD, Biochemistry
Technical Director - Clinical
Chemistry & Biochemical Genetics
NRL - Dr Lal PathLabs Ltd



MCI 15-19066

Dr. Richa Sirohi
MD, Biochemistry
Senior Consultant Biochemist
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Result/s to follow: IMMUNOTYPING, SERUM			

IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory. •Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician. •Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes. •This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor. •The report does not need physical signature.

(#) Sample drawn from outside source.

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.

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